Outlook

Urgent Recommendations for Addressing Non-Compliance with Federal Laws, Regulations, and Accreditation Standards.

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Date Tue 08-Apr-25 17:31

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Bcc Kevin Turnage <kdturnage@protonmail.com>

1 attachment (269 KB)

MFR to Executive Leadership 04082025 Signed.pdf;

Dear Members of the Governing Body and Executive Leadership,

After consultation with a legal adviser, I am sending this memorandum directly to the Governing Body because the directive at issue directly impacts your statutory and fiduciary responsibilities under federal law and the PIMC Governing Board Bylaws. As the entity responsible for policy oversight, regulatory compliance, and patient safety, the Governing Body must be fully informed of internal actions that obstruct those duties. At present, the Governing Body has not been made aware of significant issues related to noncompliant policies and procedures, nor of the obstruction of enforcement mechanisms that support accreditation and legal accountability. This communication is submitted in the interest of transparency, lawful governance, and to support your ability to fulfill your oversight obligations in alignment with Indian Health Manual policy and federal regulatory standards.

If I have inadvertently omitted any members of the Governing Body or Executive Leadership from this message, I kindly ask that it be forwarded accordingly.

Please find attached an updated memorandum titled **"Urgent Recommendations for Addressing Non-Compliance with Federal Laws, Regulations, and Accreditation Standards."** This version includes a corrected legal citation to **Indian Health Manual Part 1, Chapter 1**, which now governs official policy development, enforcement, and maintenance across IHS. The previously referenced IHM Part 1, Chapter 5 has been rescinded.

The March 21, 2025 directive issued by Chief Quality Officer Clinton Pope removes enforcement authority from the Policy Officer and obstructs federally mandated compliance functions. This directive:

- **Violates HIPAA (45 C.F.R. § 164.530(i))**, by eliminating the designated enforcement role for compliance.
- **Violates CLIA (42 C.F.R. § 493.1251)**, by disrupting oversight of laboratory and clinical policy enforcement.
- **Violates CMS Conditions of Participation**, by undermining policy integrity and facility-wide compliance.
- **Violates Indian Health Manual Part 1, Chapter 1**, which mandates that policies be actively managed, reviewed, and enforced under formal standards. By denying enforcement authority, the directive obstructs the very mechanisms required to ensure policies remain current, compliant, and aligned with regulatory standards.

The directive is also misaligned with the **IHS mission**, which prioritizes patient safety, quality of care, and public trust. Strategically, it introduces significant risk to accreditation, impairs internal controls, and erodes the culture of accountability essential for safe healthcare delivery.

As noted in the attached memo, I have formally submitted a complaint to the HHS Office of Inspector General under the **Whistleblower Protection Act**. I respectfully request prompt engagement by the Governing Body and Executive Leadership to address and correct the compliance failures detailed in the attached document.

Please confirm receipt and advise on proposed next steps.

Respectfully,
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